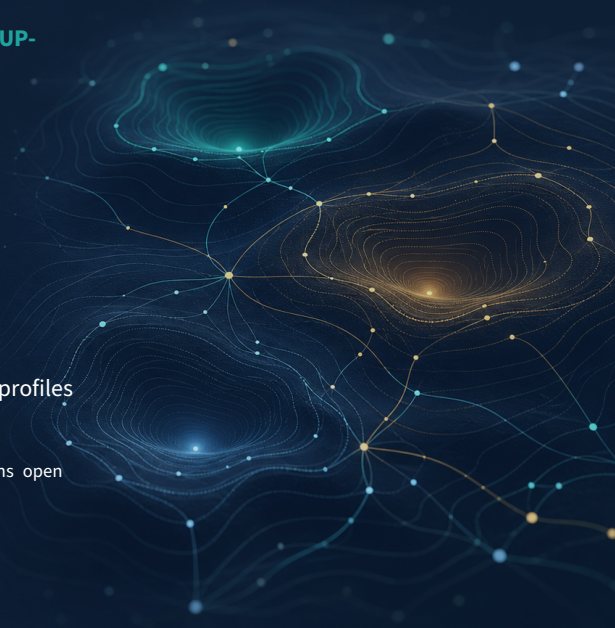
PRELIMINARY RESULTS · SCIENTIFIC ADVISORY UPDATE

FOR2107 First results

Progress toward reproducing the Tozzi framework

75 scans · 67 selected participants · first circuit profiles

23 July 2026 | What is demonstrated—and what remains open



The core idea in one minute



What Tozzi tested

Can interpretable patterns of brain-circuit function identify clinically meaningful subgroups across depression and anxiety?

Why several scan conditions matter

Rest, emotional processing, and cognitive control reveal different aspects of the same person's circuit function.

Tozzi et al. combined these readouts before defining and validating six subgroups [1].

What already works—and what does not yet

Working end to end

- ✓ All 75 resting-state scans read successfully
- ✓ Three circuit summaries calculated
- ✓ Each person compared with healthy controls
- ✓ Candidate groups created and checked
- ✓ Results and reports generated automatically
- ✓ Automated tests pass

Usable, but incomplete

- ◐ Cleaned scans accepted from outside pipeline
- ◐ Processing history still needs documentation
- ◐ No volume-by-volume motion information
- ◐ MDD/anxiety participant selection operational
- ◐ Validation currently uses short test runs
- ◐ Clinical sample is still small

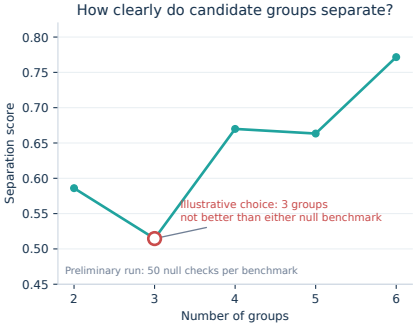
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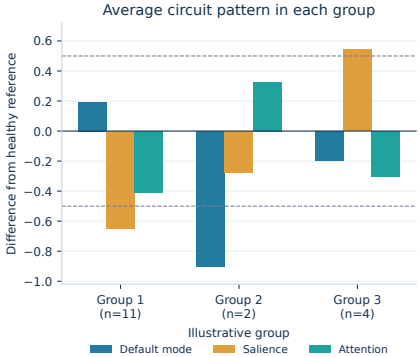
- ✗ Face-matching results
- ✗ Affective-priming results
- ✗ A direct Go/NoGo equivalent
- ✗ Independent replication sample
- ✗ Treatment-response analysis
- ✗ Final success criteria

Today: a functioning resting-state workflow. **Not yet:** a completed replication.

What the first results show

75	67	17	50	3
REST SCANS	SELECTED	CLINICAL	HEALTHY CONTROLS	CIRCUITS





What is reproducible

The complete resting-state workflow runs on real data and produces interpretable person-level circuit measurements.

What we observe

The illustrative groups have different average profiles—but two contain only two and four clinical participants.

What we cannot conclude

Group separation is not better than chance benchmarks; symptoms do not differ reliably; motion remains unassessed.

Published study versus current evidence

	Tozzi et al.	Current work	
Participants	801 clinical participants and 137 healthy controls	17 clinical participants and 50 healthy controls selected	!
Brain information	Rest, emotional processing, and cognitive control	Resting-state information only	!
Person-level profile	Multiple circuit readouts compared with healthy controls	Same principle for three available circuits	✓
How groups are found	People grouped by similarity of their profiles	Same grouping approach implemented	✓
Confidence checks	Extensive chance tests, repeated subsets, and study holdouts	Short chance tests and repeated subsets; no independent holdout	!
Clinical relevance	Differences in symptoms, behavior, and treatment response	No reliable symptom difference; behavior and treatment pending	✗
Main conclusion	Six clinically distinct multimodal subgroups	**No subgroup claim**; feasibility only	−

✓ already aligned ! only partly comparable ✗ not available − cannot assess yet

Sources: Tozzi et al. [1]; FOR2107 context [2].

What is needed for a strong scientific claim



Evidence required before replication

- Similar circuit patterns appear in the larger sample
- Groups remain stable when data and settings change
- Separation is stronger than chance
- Results survive motion and processing checks
- Groups differ on independent clinical outcomes

Decision needed now

FOR2107 has no direct Go/NoGo equivalent. We should state in advance whether the target is:

partial replication using rest + emotion **or** **scientific extension using the closest local task**

Current assessment: implementation milestone achieved · first scientific signal described · replication evidence still pending.